



Care Indeed Home Health

Mock Patient Evaluation Document · Version 1.0 MOCK · Training Date
July 1, 2026

MOCK TRAINING RECORD
SYNTHETIC UAT DATA
NOT FOR CLINICAL USE
DO NOT UPLOAD TO LIVE PATIENT
CHART

EVALUATION 2 — HIGH-PLUS ACUITY

Primary Diagnosis: Post-Surgical Bilateral Total Knee Arthroplasty with Wound Complication

PATIENT DEMOGRAPHICS

Field	Detail
Patient Name	Gerald "Jerry" Thomas Washington
Date of Birth	11/27/1952
Age	72
Sex	Male
Gender Identity	Male (he/him)
Race/Ethnicity	Black / African American / Non-Hispanic
Primary Language	English
Interpreter Needed	No
Marital Status	Married
Medical Record Number	CI-2025-00423
Address of Service	3288 Redwood Terrace, Redwood City, CA 94063
Home Phone	(650) 555-0837
Cell Phone	(650) 555-0291
Email	jerry.washington52@email.com
County	San Mateo

EMERGENCY CONTACTS

Priority	Name	Relationship	Phone (Primary)	Phone (Alt)	Lives with Patient?
1	Cynthia A. Washington	Wife	(650) 555-0838	(650) 555-0166	Yes
2	Marcus Washington	Son	(408) 555-0524	N/A	No — San Jose
3	Denise Carter	Sister	(650) 555-0710	N/A	No — Redwood City

LIVING SITUATION

Field	Detail
Lives With	Wife (Cynthia, age 70, generally healthy, active, primary caregiver)
Residence Type	Single-story ranch home, owned
Stairs to Enter	1 step (no rail) to front door; accessible ramp installed at garage entrance
Interior Stairs	None
Pets	1 small dog ("Duke"), Miniature Schnauzer, 12 lbs — active, may present tripping hazard
Home Access Instructions	Garage keypad entry; code provided to agency by wife Cynthia
DME in Home	Hospital bed (rented), bedside commode, elevated toilet seat with arms, rolling walker, front-wheeled walker (backup), CPM machine (bilateral — rented), ice therapy machine (GameReady), shower transfer bench

PAYER INFORMATION

Priority	Payer	ID / Policy Number	Group Number	Effective Date
Primary	Medicare Part A (Traditional)	3DK7-RF2-WN44	N/A	12/01/2019
Secondary	Blue Shield of California (PPO) — retiree benefit	BSC-887412309	RET-5520	01/01/2015

PHYSICIAN INFORMATION

Role	Name	Specialty	Phone	Fax	NPI
Attending / Certifying	Dr. James P. Kowalski	Orthopedic Surgery	(650) 555-0944	(650) 555-0945	3456789012
PCP	Dr. Evelyn Nakamura	Family Medicine	(650) 555-0400	(650) 555-0401	4567890123
Infectious Disease (consulting)	Dr. Priya Mehta	Infectious Disease	(650) 555-1122	(650) 555-1123	5678901234

F2F Encounter Date: 06/14/2025 (inpatient, post-op day 2) F2F Performed By: Dr. James P. Kowalski

REFERRAL / HOSPITALIZATION HISTORY

Event	Facility	Dates	Reason
Hospital Discharge	Stanford Health Care	06/12/2025 – 06/17/2025	Bilateral simultaneous TKA; complicated by right knee surgical site dehiscence (1.5 cm) on post-op day 3
Pre-op clearance	Dr. Nakamura office	05/28/2025	Cardiac and medical clearance for surgery

Start of Care Date: 06/18/2025 Referral Source: Stanford Health Care Ortho Discharge Team — Coordinator: Michael Chen, PT, (650) 555-1050

DIAGNOSES

Priority	ICD-10 Code	Diagnosis
Primary	Z96.652	Presence of bilateral artificial knee joints (status post bilateral simultaneous TKA 06/12/2025)
Surgical	M17.11, M17.12	Primary osteoarthritis, right knee; Primary osteoarthritis, left knee
Complication	T81.31XA	Disruption of surgical wound, not elsewhere classified, initial encounter (right knee dehiscence — 1.5 cm)
Complication	T84.54XA	Periprosthetic osteolysis — not currently applicable; monitoring for infection given wound issue
Secondary	I10	Essential hypertension
Secondary	E11.9	Type 2 diabetes mellitus without complications
Secondary	E66.01	Morbid (severe) obesity due to excess calories (BMI 42.3)
Secondary	G47.33	Obstructive sleep apnea (uses CPAP nightly)
Secondary	M54.5	Low back pain (chronic, lumbar DDD)
Secondary	K21.0	Gastroesophageal reflux disease with esophagitis
Secondary	F17.210	Nicotine dependence, cigarettes, uncomplicated (smoking 1/2 pack/day x 40 years; told to quit pre-surgery, reduced to 3-4 cigarettes/day)

Surgical History:

Bilateral simultaneous TKA (06/12/2025)

Right rotator cuff repair (2018)

Appendectomy (1989)

Left inguinal hernia repair (2001)

ALLERGIES

Allergen	Reaction	Severity
Codeine	Severe nausea, vomiting	Moderate
Adhesive tape	Contact dermatitis, blistering	Moderate
No other known allergies		

CURRENT MEDICATIONS

#	Medication	Dose	Route	Frequency	Purpose	High-Risk?
1	Oxycodone	5-10 mg	PO	Q4-6H PRN pain (max 40 mg/day)	Post-op pain	YES — OPIOID
2	Acetaminophen (Tylenol)	1000 mg	PO	Q8H scheduled (max 3000 mg/day with opioid)	Pain, adjunct	No
3	Gabapentin (Neurontin)	300 mg	PO	TID	Neuropathic pain / surgical pain adjunct	No
4	Enoxaparin (Lovenox)	40 mg	SubQ	Daily x 28 days post-op (through 07/10/2025)	DVT prophylaxis	YES — ANTICOAGULANT
5	Cephalexin (Keflex)	500 mg	PO	QID x 14 days (through 06/26/2025)	Surgical wound infection prophylaxis (dehiscence)	No
6	Metformin	1000 mg	PO	BID	Type 2 DM	Yes
7	Lisinopril	20 mg	PO	Daily	HTN	No
8	Amlodipine (Norvasc)	5 mg	PO	Daily	HTN	No
9	Omeprazole (Prilosec)	20 mg	PO	Daily (AM, 30 min before food)	GERD	No
10	Aspirin	81 mg	PO	Daily (HOLD during Lovenox per MD)	Cardiac — currently held	No
11	Ferrous sulfate	325 mg	PO	BID (post-op anemia — Hgb 9.2 at discharge)	Anemia	No
12	Stool softener (Docusate/Senna)	100 mg / 8.6 mg	PO	BID	Constipation prophylaxis (opioid-induced)	No
13	Nicotine patch	14 mg	Transdermal	Daily	Smoking reduction/cessation	No

Controlled Substances in Home: ☐ Yes — Oxycodone 5 mg tablets (qty 60, dispensed 06/17/2025) Sharps in Home: ☐ Yes — Lovenox prefilled syringes Sharps Container: Provided by pharmacy; in bedroom

CLINICAL ASSESSMENT — VITAL SIGNS AT ADMISSION

Parameter	Value	Abnormal?
BP	142/86 mmHg	Mildly elevated
HR	76 bpm, regular	WNL
RR	18 breaths/min	WNL
Temp	99.1°F (oral)	Borderline — monitor
SpO2	95% on RA	WNL (low-normal for this patient)
Weight	312 lbs (141.5 kg)	BMI 42.3 — morbid obesity
Height	6'1" (185.4 cm)	
Pain	7/10 bilateral knees; 8/10 right knee (at wound site)	Yes — expected post-op
Blood Glucose	204 mg/dL (2 hrs post-meal)	Elevated — stress hyperglycemia + DM

CLINICAL ASSESSMENT — WOUND EVALUATION

Right Knee — Surgical Wound with Dehiscence:

Parameter	Finding
Location	Anterior right knee, midline surgical incision
Incision Length (total)	22 cm
Wound Status	Closed with staples x 28 — EXCEPT mid-incision dehiscence
Dehiscence	1.5 cm length x 0.5 cm width x 0.3 cm depth, mid-incision
Wound Bed	80% granulation tissue (beefy red), 20% fibrinous (yellow, adherent)
Wound Edges	Irregular, slightly macerated at inferior margin; staples intact above and below dehiscence
Drainage	Moderate serosanguineous; 2x2 gauze saturated approximately 50% over 8 hours (per patient report)
Odor	None
Peri wound Skin	Erythema extends 1.5 cm circumferentially from wound edges; warm to touch; no induration, no fluctuance, no crepitus
Signs of Infection	Peri wound erythema and warmth (monitoring closely); low-grade temp 99.1°F; on prophylactic antibiotics. No purulent drainage, no foul odor, no fever >100.4°F, no sepsis symptoms.
Current Dressing	Aquacel Ag (antimicrobial) to wound bed; ABD pad secondary dressing; secured with rolled gauze and paper tape (NOT adhesive tape — allergy)
Dressing Change Frequency	Daily and PRN if saturated
MD Wound Care Orders	Cleanse with normal saline; apply Aquacel Ag to wound bed; ABD pad secondary; secure with rolled gauze and paper tape; change daily and PRN. If signs of infection worsen (expanding erythema, fever >100.4°F, purulent drainage), notify MD immediately.
Wound Photograph	☐ Taken with consent at admission
Staple Removal	Per surgeon — follow-up appointment 06/26/2025 for staple assessment

Left Knee — Surgical Wound (Uncomplicated):

Parameter	Finding
Location	Anterior left knee, midline surgical incision
Incision Length	20 cm
Wound Status	Closed with staples x 26 — all intact, well-approximated
Drainage	Minimal serous; dressing with trace staining only
Peri wound Skin	Mild ecchymosis (expected post-op), no erythema, no warmth
Signs of Infection	None
Current Dressing	Dry sterile gauze; paper tape
Dressing Change Frequency	Every other day and PRN

HOMEBOUND STATUS ASSESSMENT

Criterion	Finding
Criterion 1	MET. Patient is 5 days post bilateral TKA. Unable to ambulate more than 5-10 feet with front-wheeled walker and max assist of 1. Unable to navigate steps. Severe pain with weight-bearing (7-8/10). Morbid obesity compounds difficulty. Requires considerable and taxing effort to leave home — cannot get into a standard vehicle without significant assist.
Criterion 2	MET. Has not left home since discharge 06/17/2025. Only anticipated outings are surgeon follow-up appointments (medical care).
Homebound Status	CONFIRMED

ADVANCE DIRECTIVES

Document	Status
AHCD	Yes — executed 2020. Wife Cynthia designated as health care agent.
POLST	No
DNR	No — Full Code

HOME SAFETY ASSESSMENT

Area	Finding	Recommendation
Entry (front)	1 step, no rail	Use ramp at garage entrance exclusively until steps can be navigated
Entry (garage)	Portable ramp installed; adequate width for walker; non-slip surface	Adequate — monitor
Flooring	Hardwood throughout; 1 area rug in living room (non-slip backing)	Remove area rug (tripping hazard with walker)
Bathroom	Shower transfer bench installed; handheld showerhead present; grab bar by toilet; elevated toilet seat with arms installed	Adequate — verify stability of all equipment
Bedroom	Hospital bed in place; bedside commode; clear path to bathroom	Adequate
Lighting	Good throughout	None
Smoke/CO Detectors	Present and functional (all 4 tested)	None
Medications	Oxycodone stored in bedroom nightstand (wife manages); Lovenox in refrigerator; all other meds in kitchen cabinet	Secure controlled substance (recommend locking med box); ensure sharps container next to injection site
Oxygen	Not applicable	N/A
Electrical	CPM machine and ice therapy machine require outlets near bed; extension cord in use	Replace extension cord with heavy-duty power strip with surge protection; avoid overloading circuit
Dog (Duke)	Active; runs to door, may get underfoot with walker	Confine dog to separate room during patient transfers and ambulation; crate training recommended
Smoking	Patient smokes 3-4 cigarettes/day on back patio (seated); using nicotine patch	Reinforce smoking cessation; risk of wound healing impairment; risk of DVT; never smoke with nicotine patch on (toxicity risk); provide cessation resources
Emergency Access	Address clearly marked; wide doorways accommodate walker	Adequate

Overall Home Safety Risk Level: Moderate-High (primarily due to bilateral surgical wounds, opioid use, morbid obesity affecting safe transfers, and dog tripping hazard)

SERVICES ORDERED

Service	Discipline	Frequency	Duration
Skilled Nursing	RN	Daily x 7 days (wound care), then 3x/week x 3 weeks, then 2x/week x 4 weeks	8 weeks
Physical Therapy	PT	Daily x 2 weeks, then 5x/week x 2 weeks, then 3x/week x 4 weeks	8 weeks
Occupational Therapy	OT	3x/week x 4 weeks, then 2x/week x 4 weeks	8 weeks
Home Health Aide	HHA	Daily x 2 weeks, then 3x/week	Duration of need

SKILLED NURSING CARE PLAN FOCUS AREAS

Wound Management

Right knee: daily wound assessment with measurement, photography, and documentation. Perform wound care per MD orders (NS cleanse, Aquacel Ag, ABD pad, paper tape). Assess for signs of infection (expanding erythema, fever, purulent drainage, wound culture if indicated). Mark erythema borders with skin marker to track changes.

Left knee: assess every other day; dressing change per orders.

Staple line assessment bilaterally each visit.

Coordinate with surgeon re: staple removal timeline and wound closure plan.

Infectious disease consult follow-up — ensure cephalexin compliance and assess response.

DVT Prevention & Monitoring

Enoxaparin 40 mg SubQ daily — assess injection sites (rotate abdomen), teach patient/wife injection technique for days RN not present.

Assess for DVT signs each visit: unilateral calf swelling, warmth, tenderness, Homans' sign, new-onset dyspnea (PE concern).

Reinforce ankle pumps, early mobility per PT plan.

Monitor platelets (HIT risk with enoxaparin — lab draw per MD schedule).

Pain Management

Assess pain using numeric rating scale (0-10) at each visit — bilateral, separate scores for each knee.

Monitor opioid use: quantity tracking, assess for side effects (constipation, sedation, respiratory depression, urinary retention), educate on safe use.

Opioid tapering plan: coordinate with surgeon. Goal: transition from oxycodone to non-opioid analgesics within 3-4 weeks.

Assess for opioid misuse risk factors; use SOAPP or equivalent screening tool.

Non-pharmacologic pain management: ice therapy (GameReady), elevation, relaxation techniques, TENS if ordered.

Diabetes Monitoring

FSBS monitoring: BID (AM fasting + 2 hr post-meal) — anticipate stress hyperglycemia post-surgery.

Coordinate with PCP for potential insulin coverage or metformin dose adjustment.

Wound healing risk: educate on glycemic control's critical role in wound healing.

Smoking Cessation

Educate on impact of smoking on wound healing (vasoconstriction, hypoxia, increased infection risk) and DVT risk.

Nicotine patch compliance monitoring (reinforce: do NOT smoke while patch is on).

Motivational interviewing techniques; referral to California Smokers' Helpline (1-800-NO-BUTTS).

Post-Op Education

Teach signs of surgical site infection.
Teach signs of DVT/PE — seek emergency care immediately for sudden-onset dyspnea, chest pain, unilateral leg swelling.
Total knee precautions (per surgeon protocol).
Weight-bearing status and progression (per PT/surgeon).

PHYSICAL THERAPY FOCUS AREAS

ROM Progression — Bilateral knee flexion/extension; CPM machine use (settings per surgeon protocol — starting 0-90°, advance per tolerance)

Weight-Bearing Progression — Per surgeon: weight-bearing as tolerated (WBAT) bilateral with walker

Gait Training — Progressive ambulation with walker; transition to cane when appropriate; stairs when safe

Strengthening — Quad sets, straight leg raises, hamstring sets, ankle pumps → progressive closed chain exercises

Transfer Training — Bed mobility, sit-to-stand (high surfaces initially due to TKA precautions and BMI), car transfers (for MD appointments)

Balance — Static → dynamic balance progression; fall prevention

Bariatric Considerations — All interventions modified for 312 lb body weight; appropriate DME weight ratings verified; safe manual handling techniques

PLAN OF CARE GOALS (60-DAY CERTIFICATION PERIOD)

#	Goal	Target
1	Right knee wound dehiscence fully closed with no infection	07/31/2025
2	Left knee incision fully healed, staples removed per surgeon	07/10/2025
3	Patient/wife demonstrate Lovenox injection technique independently	06/25/2025
4	Bilateral knee flexion $\geq 110^\circ$	08/17/2025
5	Bilateral knee extension 0°	08/17/2025
6	Ambulate 300 feet with least restrictive device and no more than standby assist	08/17/2025
7	Transfer bed/chair/toilet independently	08/01/2025
8	Negotiate stairs (4 steps with rail) independently	08/17/2025
9	Pain controlled at $\leq 4/10$ with non-opioid regimen	08/01/2025
10	Zero DVT/PE events	08/17/2025
11	Fasting blood glucose consistently < 180	08/17/2025
12	Smoking reduced to 0 cigarettes/day	08/17/2025
13	Zero falls	08/17/2025
14	ADL independence (bathing, dressing, toileting)	08/17/2025

PATIENT SIGNATURES — EVALUATION 2

Patient Signature: _____ Date: _____

Print Name: Gerald Thomas Washington

Caregiver/Wife Signature: _____ Date: _____

Print Name: Cynthia A. Washington

Clinician Signature: _____ Date: _____

Patient / Representative / Conservator

Mock signature area — training only.

Evaluating Clinician

Mock signature area — training only.

Caregiver / Interpreter if applicable

Mock signature area — training only.

Date / Attestation

Mock signature area — training only.